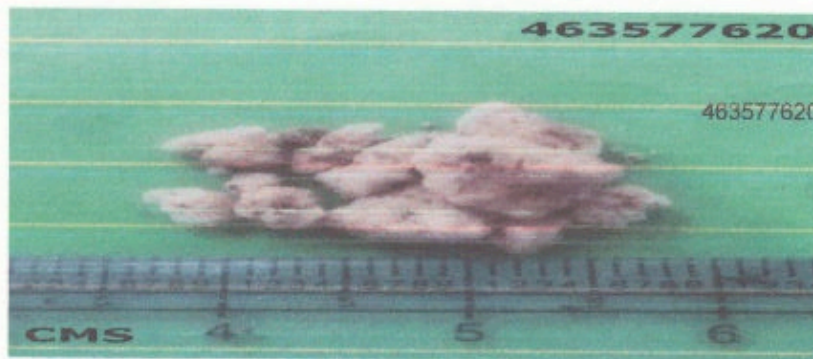


Name	: Mr. GANESH DASS	Age	: 31 Years
Lab No.	: 463577620	Gender	: Male
Ref By	: DR ADEEB SINGH	Reported	: 21/12/2024 4:33:18PM
Collected	: 19/12/2024 1:44:00PM	Report Status	: Revised
A/c Status	: P		
Collected at	Hexamed Healthcare Services LLP 25-270/6, Taluka Bahu, Main Bypass Road, NH1A, Channi Himmat, Jammu,		

Test Report

STONE ANALYSIS BY FTIR



Sample: Kidney Stones

Aggregate Weight: 0.759 gm

Composition : Calcium Oxalate Monohydrate 50% Calcium Oxalate Dihydrate 50%

(Representative stone analysed)

Kidney stone (Nephrolithiasis) develops when urine becomes supersaturated with crystal forming substances such as Calcium, uric acid, oxalate etc.(resulting from dehydration or genetic predisposition) or due to decrease in urinary concentration of substances such as magnesium, citrate etc. that inhibit stone formation. Kidney stones tend to recur; approximately 50% of patients who form one stone form another in 10 years. Nephrolithiasis is considered to be a disease of affluence because it is more prevalent in conditions like Obesity, Hypertension & Diabetes. For patients with multiple kidney stones 24-hour urine chemistries should be measured to identify abnormalities that predispose to kidney stones, which guides dietary and pharmacologic therapy to prevent future stone events.

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Test Report

STONE TYPE	FREQUENCY(%)	CAUSES
Calcium oxalate / mix	75	- Hypercalciuria with Hypercalcemia (Hyperparathyroidism, Vitamin D excess, malignancy) - Idiopathic hypercalciuria - Hypocitraturia - Alkaline urine, low urine volume - High dose of vitamin C - Metabolic syndrome - Inflammatory bowel disease
Calcium Phosphate	5	- Distal Renal Tubular acidosis - Primary Hyperparathyroidism - Chronic urinary tract infection - Hypercalciuria and/or Hypophosphaturia
Magnesium Ammonium Phosphate (Struvite)	10-20	Recurrent urinary tract infection
Uric Acid	5-10	- Hyperuricosuria and /hyperuricosemia - Metabolic syndrome (Diabetes, obesity, Hypertension) - Acidic urine (pH<5.5) - Gout
Cystine	1	Cystinuria (A genetic disorder)

DIETARY RECOMMENDATIONS FOR PREVENTION OF RECURRENCE

General Dietary measures:

- Fluid intake should be between 2.5-3L/day or it should be sufficient to produce urine output of 2L/day
- Diet rich in fibres, fruits (orange, lemon, apple etc) & vegetables
- Maintain adequate calcium intake (1000-1200 mg/day)
- Aim to achieve/maintain healthy weight
- Reduce intake of animal protein (meat & dairy)

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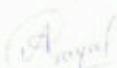
Report Status : Revised

Test Report

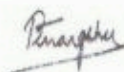
Specific measures based on metabolic evaluation and stone composition:

STONE COMPONENT	METABOLIC PROFILE	SUGGESTED DIETARY RECOMMENDATION
Calcium oxalate or combined calcium oxalate/phosphate stone	Hypercalciuria	Salt restriction
	Hyperoxaluria	Calcium supplements with meal Restrict items rich in oxalate eg. Soya protein, Spinach, Nuts, Strawberries, Cocoa, Chocolates,
		beetroot, carrot, radish etc
	Hyperuricosuria	- Limit animal protein & purine intake (Fish, lamb, beef, chicken, pork etc), cauliflower, Mushroom - Reduce alcohol intake
	Hypocitraturia	Increase citrate rich food (Orange, lemon, apple etc)
Uric Acid	Hyperuricosuria & Hyperuricemia	- Limit animal protein & purine intake (Fish, lamb, beef, chicken, pork etc), cauliflower, Mushroom - Reduce alcohol intake
	Metabolic syndrome	Lifestyle changes, weight loss, Diabetes control
Cystine		- Increase fluid intake to 3-4L, Urine output should at least be 3L - Restrict sodium & protein intake

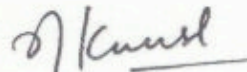
This is an amended report which supercedes the previuos report against same lab no.


 DMC - 67327

 Dr Anjalika Goyal
 MD, Biochemistry
 Consultant Biochemist


 DMC - 89819

 Dr Himangshu Mazumdar
 MD, Biochemistry
 Sr. Consultant Biochemist


 DMC - 9550

 Dr Nimmi Kansal
 MD, Biochemistry
 Technical Director - Clinical Chemistry
 & Biochemical Genetics

This is a revised report & supersedes all the previously issued reports

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